BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks	Full blood count, kidney and liver tests, a kidney filtration assessment to calculate carboplatin, and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before the first fluorouracil unless already tested. Measured kidney filtration is preferred where feasible. Hearing assessment is arranged when indicated. Confirm recovery from chemoradiotherapy, swallowing, weight and intake.	Full blood count, kidney and liver tests before each cycle. Review mouth soreness, swallowing, weight, intake and painful palms or soles; hearing and nerve symptoms also need review. If kidney function changes, the carboplatin calculation may need reassessment.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Continue recovery and follow-up after chemoradiotherapy and chemotherapy, including mouth, dental, swallowing, nutritional, hearing or nerve problems. Examinations, scans and any tumour-specific tests follow the agreed nasopharyngeal cancer plan; this regimen does not prescribe a universal interval.
Treatment and appointments	Confirm the diagnosis, aim, prescription and actual calendar.	Carboplatin is infused over 30 minutes on Day 1. Its dose is calculated from kidney function using a target exposure called AUC 5. Fluorouracil 1,000 mg/m² per day is given as a continuous 96-hour infusion, or as the source-listed daily 22-hour option on Days 1–4. Your unit confirms the actual prescription, pump changes and disconnection time.	Follow the written home plan and keep pump or line appointments where applicable.	After recovery, adjuvant chemotherapy is planned to begin 4 weeks after completion of weekly carboplatin with radiotherapy (National Cancer Control Programme (NCCP) 00419). Carboplatin is given on Day 1 and fluorouracil over Days 1–4, every 28 days for up to 3 cycles. The treating team confirms readiness and dates.	Record the last dose and follow-up appointments.
Mouth, weight and swallowing	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.
Cancer assessment	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00552 v6: official regimen

Tests in your plan

Full blood count

WHEN

At baseline and before each cycle

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and before each cycle

Guide safe treatment and identify organ problems.

DPD assessment

WHEN

Before the first fluorouracil treatment unless already tested

Checks for reduced ability to break down fluorouracil; results affect treatment choice or dose. Testing cannot exclude every severe reaction. Kidney impairment can affect one blood-based test, so the team chooses the appropriate method.

Kidney filtration assessment

WHEN

Before treatment; reassessment when kidney function or the clinical situation changes

Carboplatin dose is calculated from kidney filtration. A measured test is preferred where feasible; the team chooses a valid method and reassesses significant changes.

Hearing assessment

If part of your plan

WHEN

At baseline when clinically indicated; review new hearing or balance symptoms during treatment

Investigates hearing loss or tinnitus and helps judge platinum-related risk.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Carboplatin + fluorouracil after chemoradiotherapy · nasopharyngeal cancer: treatment and checks

Adjuvant chemotherapy after chemoradiotherapy for locally advanced stage III or intravenous (IV) nasopharyngeal carcinoma when cisplatin is contraindicated or not tolerated.

After recovery, adjuvant chemotherapy is planned to begin 4 weeks after completion of weekly carboplatin with radiotherapy (NCCP 00419). Carboplatin is given on Day 1 and fluorouracil over Days 1–4, every 28 days for up to 3 cycles. The treating team confirms readiness and dates.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests, a kidney filtration assessment to calculate carboplatin, and DPD testing before the first fluorouracil unless already tested. Measured kidney filtration is preferred where feasible. Hearing assessment is arranged when indicated. Confirm recovery from chemoradiotherapy, swallowing, weight and intake.	Full blood count, kidney and liver tests before each cycle. Review mouth soreness, swallowing, weight, intake and painful palms or soles; hearing and nerve symptoms also need review. If kidney function changes, the carboplatin calculation may need reassessment.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Continue recovery and follow-up after chemoradiotherapy and chemotherapy, including mouth, dental, swallowing, nutritional, hearing or nerve problems. Examinations, scans and any tumour-specific tests follow the agreed nasopharyngeal cancer plan; this regimen does not prescribe a universal interval.
Treatment and appointments NCCP	Confirm the diagnosis, aim, prescription and actual calendar.	Carboplatin is infused over 30 minutes on Day 1. Its dose is calculated from kidney function using a target exposure called AUC 5. Fluorouracil 1,000 mg/m² per day is given as a continuous 96-hour infusion, or as the source-listed daily 22-hour option on Days 1–4. Your unit confirms the actual prescription, pump changes and disconnection time.	Follow the written home plan and keep pump or line appointments where applicable.	After recovery, adjuvant chemotherapy is planned to begin 4 weeks after completion of weekly carboplatin with radiotherapy (NCCP 00419). Carboplatin is given on Day 1 and fluorouracil over Days 1–4, every 28 days for up to 3 cycles. The treating team confirms readiness and dates.	Record the last dose and follow-up appointments.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Mouth, weight and swallowing NCCP	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.
Cancer assessment NCCP	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

What each check is for

1 Adjuvant chemotherapy after chemoradiotherapy

Up to 3 cycles · every 4 weeks

This chemotherapy is an additional treatment after chemoradiotherapy for nasopharyngeal cancer. It aims to treat cancer cells that may remain. The team explains the expected benefit and burden for you, including recovery from the earlier treatment.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.47